
Joint & Bursa Injections: Patient Information Leaflet

DUROLANE (Hyaluronic Acid) vs. Corticosteroids vs. PRP for Joint & Bursa Injections

This leaflet has been designed to help you understand the three main types of injections used to treat pain in joints and bursae (fluid-filled sacs that cushion bones and tendons). It also includes specific information for injections around the shoulder (SASD bursa), hip (GT bursa), and sitting bones (ischial bursa). Your doctor has recommended an injection to help manage your condition. Please use this information to discuss which option is best for you.

A separate leaflet on utility of PRP injections is available on our website

What is the difference?

Think of it this way:

- **Corticosteroids** are like fire extinguishers. They are powerful and work quickly to put out the inflammation (the "fire").

• **Durolane** is like engine oil. It lubricates the moving parts and helps them glide smoothly, protecting them from wear and tears. ***Patients have to purchase the injection or get it covered by insurance.***

• **PRP** is like a repair crew. It uses your body's own healing cells to stimulate repair and regeneration of damaged tissues.

The Comparison: A Quick Overview

	Corticosteroids	Durolane	PRP
What are these?	These are powerful, fast-acting anti-inflammatory medications	Is a lubricating and shock-absorbing gel, similar to healthy joint fluid	Is a concentration of your own healing cells, taken from a sample of your blood
Main Goal	To rapidly reduce pain, swelling, and inflammation	To improve joint lubrication, cushion the joint, and reduce friction pain	To stimulate the body's natural healing process, potentially repairing damaged tissue
General Use	Best For Inflammatory flare-ups. Ideal for hot, swollen, and very painful joints or bursae (e.g., bursitis)	Best for chronic "wear and tear" joint pain where natural fluid has thinned	Tissue repair, ideal for mild to moderate arthritis, chronic tendonitis, and injuries where healing is slow

	Corticosteroids	Durolane	PRP
How is it given?	A single injection	A single injection	A single or a series of injections. Blood is drawn, processed, and injected in the same visit
How long until it works?	Usually, 2- 4 weeks	2 to 4 weeks after the injection	4 to 6 weeks (or longer) as the body needs time to heal
How long until it works?	Weeks to a few months. Does not repair damage	6 to 12 months. Improves the joint environment.	12 to 18 months or longer. Aims to repair the underlying tissue

Side effects of Corticosteroids

Corticosteroids over repeated injections have a lot of side effects, please refer to the general information section on the website and you will find a detailed Patient Information Leaflet on Risks of Corticosteroids.

- Temporary pain increases.
- Skin thinning/lightening at site.
- Raises blood sugar.
- Limited repeats per year (can weaken cartilage over time).



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Side effects of Durolane

- Mild, temporary pain, swelling, or warmth.
- Very safe for repeat use.
- Can be given once a year.

Side effects of PRP

- Temporary increase in pain or stiffness for a few days.
- Very safe (uses your own cells).
- Minimal risk of reaction.

Information on Specific Bursa Injections

This small, fluid-filled sac sits in your shoulder, just under a bony part of your shoulder blade. It can become inflamed, causing pain when you lift your arm.

Corticosteroids: Research shows that a single, guided injection of steroid into this bursa can significantly improve shoulder pain and movement for up to 3 months. It is a very effective treatment for flare-ups.

Durolane & PRP: These are not typically the first choice for this specific bursa. Treatment usually focuses on the underlying tendon problem.



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Greater Trochanteric Bursa (Hip)

This bursa is located on the outside of your hip. Pain here is often part of a condition called Greater Trochanteric Pain Syndrome (GTPS), which can affect the bursa and the nearby tendons.

Corticosteroids: Offers fast, effective relief for sudden, painful flare-ups of hip bursitis.

PRP: The evidence is mixed, but promising. Some studies suggest PRP may provide better long-term pain relief than steroids for chronic GTPS. Your doctor will discuss if you are a suitable candidate.

Durolane: There is some evidence for using Durolane in this specific bursa.

Ischial Bursitis (Sitting Bone Pain)

This bursa is located deep in your buttock; over the bone you sit on. When inflamed, it causes pain when sitting for long periods.

Corticosteroids: Can be very effective, but accuracy is key. Research shows that using imaging guidance (like ultrasound or X-ray) to place the injection precisely into the bursa provides significantly better pain relief (around 86% improvement) compared to injections placed by touch alone (around 55% improvement).

Durolane & PRP: There is currently very little published evidence for using Durolane or PRP specifically for ischial bursitis.

Which One Might Be Right for Me? A Summary

Your doctor may recommend **CORTICOSTEROIDS** if:

- You have a sudden, painful flare-up of bursitis (shoulder, hip, or sitting bone).
- Your joint or bursa is hot and swollen.
- You need rapid pain relief to start physical therapy.
- You have ischial bursitis and can have an image-guided injection for the best result.

Your doctor may recommend **DUROLANE** if:

- You have mild to moderate osteoarthritis ("wear and tear" arthritis) in a joint like the knee or hip.
- You are looking for longer-term relief (6-12 months) rather than a quick fix.
- You want to delay or avoid joint replacement surgery.
- You have diabetes and need to avoid steroid-related blood sugar spikes.

Your doctor may recommend **PRP** if:

- You have Tennis Elbow, Knee Arthritis, Patellar Tendinopathy (knee), Rotator Cuff Tendinopathy (shoulder) and Plantar Fasciitis (foot). Evidence suggests PRP may offer long-term benefits.
- You have Greater Trochanteric Bursitis (GTPS) PRP may be superior to Corticosteroids.
- You are looking for treatment that uses your body's own healing powers for potentially the longest-lasting result.



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Important Considerations

Combination Therapy: Sometimes, a steroid is used first to calm a flare-up, followed by Durolane or PRP later for longer-term joint health.

Accuracy Matters: For ALL injections, especially deep ones like the ischial bursa, using ultrasound or X-ray guidance helps ensure the medication goes exactly where it's needed, leading to better results. At Red Sands we always perform guided injections.

Part of a Plan: These injections work best when combined with physical therapy, exercise, and activity changes.

Frequently Asked Questions

Q: Will the injection hurt?

A: Most people feel a little sting from the local anesthetic used to numb the area. You may feel pressure as the medication is injected. For PRP, there is also the initial pinch of blood drawn. Your doctor will do this as quickly and comfortably as possible.

Q: What should I do after the injection?

A: You will likely be advised to rest the area for 24-48 hours. Avoid strenuous activity. For PRP, you may need to avoid anti-inflammatory medication (like Ibuprofen) as it can interfere with healing. Your doctor will give you specific instructions.



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Q: When should I call the doctor?

A: Contact your doctor if you have severe pain that gets worse, not better; signs of infection (increasing redness, warmth, fever); or any other concerning symptoms.

Contact Your Care Team Before/After Procedure:

Phone: 782-377-7813



Info@redsandspm.com

In an emergency:

→ Call 911, or visit your local emergency department

Disclaimer

This information is for general guidance only. It is not a substitute for professional medical advice. Always follow the specific instructions given by your healthcare provider and discuss any questions or concerns you have about your treatment.